

Policy on Ethical Management of the Organisation

«Hospital Name»

Document control

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Prepared by	Medical Superintendent Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

The hospital runs on clear ethical principles. Everyone in the hospital knows what those principles are, how to raise a concern, and that the hospital will not hide who owns it or overstate what it has achieved.

1. Purpose

This policy explains how «Hospital Name» builds and maintains its ethical management framework — the principles, codes of conduct and processes that guide the hospital to act honestly in everything it does. It covers how ethical concerns and dilemmas are raised and resolved, how the hospital discloses its ownership, and how it represents its affiliations and accreditations truthfully.

This policy does not cover patient assessment, clinical care, medication management, patient rights, infection control, or quality and safety monitoring — those are covered in other hospital policies.

2. Scope

This policy applies to those responsible for governance, the person leading the organisation, the Medical Superintendent, department heads, and the Quality Coordinator at «Hospital Name». It also applies to anyone who handles ethical concerns, prepares or reviews public information about the hospital's ownership, affiliations or accreditations, or manages conflicts of interest.

3. Policy standards

«Hospital Name» keeps a written ethical management framework and uses it. The framework states the hospital's guiding ethical principles and codes of conduct, gives staff a clear path for raising and resolving ethical concerns — clinical or non-clinical — within a defined time frame, and records every decision made. The hospital publicly discloses who owns it and only claims the affiliations and accreditations it actually holds.

Staff follow the written guidance below, keep the records it requires, and raise anything not being done to the Medical Superintendent.

4. Non-negotiable rules

1. Do not run the hospital without a written ethical management framework. The framework must state the hospital's ethical principles, cover codes of conduct, and exist as a standing document — a set of resolutions buried in meeting minutes does not satisfy this rule.
 2. Do not leave any ethical concern — clinical or non-clinical — without a defined path to resolution. Staff must know where to take it and how long it should take to be decided.
 3. Do not leave an ethical issue open past the time frame the framework sets. Decisions are documented and corrective action follows within the period the framework names.
 4. Do not hide or misrepresent who owns the hospital. Ownership is publicly disclosed in accordance with the legal form under which the hospital is registered.
 5. Do not misrepresent an affiliation or accreditation the hospital holds — what it covers, its current status, or which parts of the hospital it applies to.
 6. Staff who see any of these rules broken report it the same shift to the department in-charge or the Medical Superintendent.
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5. What we do

5.1 Keep a written ethical management framework

«Hospital Name» maintains a written ethical management framework that states how the hospital behaves and makes decisions when ethical questions arise. Transparency is a core guiding principle. The framework covers codes of conduct for staff — for doctors, the Medical Council of India’s Code of Medical Ethics, 2002 is a recognised minimum reference — and addresses areas such as handling complaints and grievances, clinical care delivery, and research. The framework names who is responsible for it, carries a dated approval, and is reviewed at defined intervals.

This framework is the hospital’s ethical foundation, not a list of aspirations. A set of good intentions captured only in meeting minutes does not meet this requirement.

5.2 Give staff a clear path for raising and resolving ethical issues

The framework includes a documented process for managing ethical issues, dilemmas and concerns. Issues may be clinical or non-clinical. Examples include conflicts of interest, breaches of confidentiality, questions about informed consent, and decisions about whether to continue or withdraw life-sustaining treatment.

The hospital applies the same approach to all such situations, based on the framework’s guiding principles. Staff are given practical guidance on what to do when an ethical question arises. The process collects, logs and analyses ethical dilemmas so that corrective and preventive action can be taken within the time frame the framework sets. Every decision is documented, and the hospital reviews completed cases for opportunities to improve.

5.3 Disclose who owns the hospital, publicly

«Hospital Name» discloses its ownership publicly and in accordance with the legal form under which it is registered — whether under the Companies Act, a Charitable Trust Act, the Societies Registration Act, the Limited Liability Partnership Act, as a sole proprietorship, or under government ownership. The disclosure may take the form of a registration certificate on display, a statement at reception, or publication on the hospital’s website. The Medical Superintendent confirms that the disclosure is current at least once a year.

5.4 Represent affiliations and accreditations honestly

«Hospital Name» accurately represents its affiliations and accreditations in all public-facing materials. Where an accreditation or affiliation applies to a specific department or service, it is described as such — not as a whole-hospital achievement. Applied-for accreditations are not described as awarded. Accreditations or affiliations that have lapsed are removed from public display. The Quality Coordinator reviews public materials periodically to confirm this.

6. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that this policy is followed and properly resourced. Acts as the head of the organisation unless the governing body has named someone else.
Medical Superintendent	Owns the day-to-day records and implementation for this policy.
Governing body	Supports the ethical management framework; receives ethics matters that reach governance level; records decisions taken on ethics issues raised.
Quality Coordinator	Audits this policy periodically; keeps training records; reviews public materials for affiliation and accreditation accuracy.
Department heads	Carry out the department-level duties this policy names.

7. Quality monitoring

The Quality Coordinator audits this policy periodically, checking a sample of records against the steps in Section 5, confirming documentary evidence is on file for each asterisked objective element in the sample (marked * in the Level column of Section 11), and confirming the CORE objective element shows no critical gaps.

If an audit finds a gap that is still open after a defined timeframe, the hospital carries out a root-cause analysis.

This policy is reviewed every year, and sooner if the ethical management framework is changed, a significant ethical issue arises, or the hospital's ownership or accreditation status changes.

8. Training and staff acknowledgement

Staff covered by this policy are trained when they join, and again every year after that. Training covers what is in Section 5 and the non-negotiable rules.

Staff acknowledgement

I have read the Policy on Ethical Management of the Organisation of «Hospital Name». I will follow the processes described.

Name	
Designation	
Department / floor	
Date	
Signature	

(One row per staff member. The Quality Coordinator holds signed acknowledgements with the induction record.)

9. Distribution

Medical Superintendent; governing body; Quality Coordinator; department heads; staff covered by this policy.

10. Abbreviations

Abbreviation	Meaning
CORE	Core objective element (NABH)
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OE	Objective Element
ROM	Responsibilities of Management (NABH Hospitals 6th Edition chapter)

11. Traceability to NABH HCO Full Accreditation 6th Edition ROM.2

This table is an index. It is not how the policy is organised.

An asterisk in the Level column means documentation of the process is required.

OE	Level	Requirement	Where	Responsible
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			addressed	
ROM.2.a	CORE*	The leaders establish the organisation's ethical management framework.	Section 3; 5.1	Medical Superintendent
ROM.2.b	Commitment*	The ethical management framework includes processes for managing issues with ethical implications, dilemmas and concerns.	Section 3; 5.2	Medical Superintendent
ROM.2.c	Commitment	The organisation discloses its ownership.	Section 3; 5.3	Medical Superintendent
ROM.2.d	Commitment	The organisation honestly portrays its affiliations and accreditations.	Section 3; 5.4	Medical Superintendent

12. Required Records / Evidence Checklist

Records the hospital holds under this policy, listed by objective element.

ROM.2.a — The leaders establish the organisation's ethical management framework.

- The written ethical management framework: stated principles, codes of conduct, scope of areas covered, and named owner.
- Dated approval record for the framework.
- Evidence the framework exists as a standing document, not meeting minutes only.

ROM.2.b — The ethical management framework includes processes for managing issues with ethical implications, dilemmas and concerns.

- Documented process for raising and resolving ethical issues, with a named time frame for resolution.
- Log of ethical issues raised, with decision and date for each.

- At least one example case record showing the process was actually followed.

ROM.2.c — The organisation discloses its ownership.

- Current ownership-disclosure text consistent with the hospital's legal form of registration.
- Evidence of public display — reception board, registration certificate on display, website page, or equivalent.
- Annual check record confirming the disclosure is current.

ROM.2.d — The organisation honestly portrays its affiliations and accreditations.

- Current list of affiliations and accreditations included in public materials.
- Periodic review record confirming public materials accurately represent what is held, its scope and its current status.
- Correction record for any misrepresentation found and fixed.

13. References

- NABH Accreditation Standards for Hospitals, 6th Edition — standard ROM.2 (official portal PDF).
 - NABH Guidebook to Accreditation Standards for Hospitals, 6th Edition — ROM.2 interpretations.
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Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked [Hospital to define] must be replaced with the hospital's own decision; a document issued with those markers left in place is not an approved policy.

The requirements in this document are accreditation requirements of the NABH Full Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. This policy does not import the Consumer Protection Act, 2019, the Clinical Establishments Act, 2010, or the Mental Healthcare Act, 2017 as a checklist. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

The clinical and technical content reflects recognised national and international guidance current at the date of preparation. «Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed.

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